National Cancer Control Programme (NCCP) 00316 · clinician draft

Dose-dense AC → fortnightly paclitaxel + weekly trastuzumab

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

AC · 14-day schedule

Active medicines: Doxorubicin + Cyclophosphamide

Emetogenic risk: High · AC

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 No routine Days 2–4 dexamethasone in this National Cancer Control Programme (NCCP) AC antiemetic row
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, interactions and glucose
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications

Additional phase-specific support

- This AC antiemetic script is replaced at the taxane transition.
- Growth-factor support is required for dose-dense AC. Specify the product, dose and days under local policy.
- Select the taxane support plan at the phase transition; do not continue the AC antiemetic course by default.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00316 v6 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.18 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 1 of each 14-day taxane block · paclitaxel + trastuzumab

Active medicines: Paclitaxel + Trastuzumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Fortnightly paclitaxel: dexamethasone 20 mg orally approximately 12 and 6 hours before treatment OR 20 mg intravenous (IV) 30 minutes before, plus chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before. Select one steroid route, not both.
- The consultant may reduce or omit steroids after treatment without hypersensitivity. If aprepitant is added, consider dexamethasone 12 mg on treatment day under the source/local plan.
- Growth-factor support is required for the dose-dense paclitaxel cycles; specify the product and days.
- Trastuzumab is weekly during this fortnightly paclitaxel phase. Antibody-only Day 8 does not require paclitaxel premedicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00316 v6 · p6 · NCCP antiemetics v8, Table 1 pp4–5

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Day 8 of the taxane block · trastuzumab only

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Paclitaxel is not due on this Day 8. Do not repeat paclitaxel premedicines or its antiemetic order.
- Continue the weekly trastuzumab schedule and cardiac plan; this is not yet the q21-day continuation phase.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00316 v6 · p6 · NCCP antiemetics v8, Table 1 pp4–5

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HER2-only continuation

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine chemotherapy antiemetics or paclitaxel premedicines carry into trastuzumab-only continuation. Review previous symptoms/reactions individually.
- Continue the cardiac-monitoring clock and confirm the prescribed route, interval and missed-dose rules.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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